YourCounselor

COUNSELLOR PRACTICE HANDOUT

Case History and Mental Status Examination

A practical interview map, note-taking worksheet and MSE practice guide for psychologists, professional counsellors and supervised trainees. Use alongside the service's approved record system and local policy. This handout is not a diagnostic instrument, risk prediction tool, or substitute for training and supervision.

Using this handout

Ask only for information that is relevant to the client's goals, safety, referral question and service. Explain why sensitive questions may matter. The client may ask for a pause, clarification or a different pace. Record the source of information and distinguish what was reported, observed and inferred.

> PRACTICE BOUNDARIES
>
> Follow local consent, privacy, safeguarding, record-keeping and emergency procedures. A recorded thought of harm in an MSE does not replace a fuller, context-based safety assessment. Sudden changes in alertness or cognition may need prompt medical attention.

A simple interview sequence

1. **Orient.** Explain role, service, consent, confidentiality limits, records and practical arrangements.
2. **Begin with the client's priorities.** Invite the client's account before moving into structured topics.
3. **Explore relevant context.** Ask about timeline, functioning, health, support and strengths at a pace the client can manage.
4. **Attend to safety.** Ask directly when indicated, clarify the context and agree concrete next steps.
5. **Summarise and plan.** Check your understanding, name what remains unknown and agree who will do what and when.

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Case-history interview map

Use the prompts selectively. They are invitations, not a mandatory checklist.

Area	Consider
Referral and purpose	Who initiated contact? What question prompted it? What does the client want from the meeting? Clarify role and service limits.
Current concern and timeline	Client's words, onset, course, frequency, intensity, triggers, exceptions, impact and what has already been tried.
Daily functioning and needs	Relationships, work or study, self-care, sleep, appetite, energy, concentration, valued activities, disability and access needs.
Health and previous support	Relevant physical health, medicines and supplements, substance use, prior care, helpful or unhelpful experiences, and possible medical review.
Development, education and work	Developmental, learning, education or work history only where it helps answer the referral question or client's goal.
Relationships and social context	Important relationships, caregiving, housing, finances, discrimination, legal or migration pressures, support and safety.
Culture, identity and values	Client's own language, cultural and community context, identity, faith, spirituality and values. Ask; do not assume.
Strengths, coping and hopes	What has helped before, what matters, supportive people, practical resources and what progress would look like.
Safety and safeguarding	Relevant immediate safety concerns, harm to self or others, abuse, coercion, neglect or risks to dependants. Follow local policy and complete fuller assessment when indicated.

Example prompt: "What would you most like help with, and what feels important for me to understand first?"

Documentation: Separate the client's account, information from another source, direct observation and practitioner interpretation. Note unanswered questions, uncertainty, consent and agreed follow-up. Collect only relevant identifying detail.

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Case-history note-taking worksheet

Use the service's approved system for identifiable clinical records. Do not store completed worksheets in an unsecured location.

Client identifier or approved record reference

Date

Practitioner and role

Setting or mode

Client's priorities and referral question

What does the client want help with? What is the referral question?

Current concern and timeline

Client's description, onset, pattern, triggers, impact and what has helped or worsened it.

Functioning, health and previous support

Relevant daily functioning, physical health, medicines, substance use, past support and access needs.

Relationships, context and strengths

Relevant relationships, culture, values, housing or work context, protective supports and coping.

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Safety and safeguarding

What was asked? What was reported? What remains uncertain? What action or consultation is needed?

Summary and follow-up

What is known? What is not yet assessed? Agreed actions, responsible person and review time.

Information source, consent and record handling

Who provided each relevant detail? What consent or lawful basis applies to any information sharing? Where will the approved record be stored?

Action tracker

Action	Owner	Due or review time

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Mental status examination

A focused description of current presentation from interview and observation. Tailor it to the referral question and the practitioner's competence.

Domain	Describe rather than label	Example
Appearance	Relevant observable presentation; avoid assumptions about character or diagnosis.	Clean work clothes; appeared tired.
Behaviour and engagement	Interaction, engagement, rapport and context. Consider culture, trauma, disability, neurodiversity and preference.	Engaged; looked away while thinking.
Alertness	Ability to participate; note unusual drowsiness, confusion or fluctuation.	Alert and responsive throughout.
Motor activity	Observable pace or movement, with context.	Shifted position frequently while discussing work.
Speech and communication	Rate, volume, amount, articulation, fluency and latency; note access needs.	Clear, normal volume, pauses before answering.
Mood and affect	Mood is reported; affect is observed expression. Describe separately.	Mood "worn out"; affect subdued and reactive.
Thought process	Organisation, coherence, pace and goal direction.	Relevant and generally linear.
Thought content	Themes in the client's report, including meaning, distress and impact. Attribute to client.	Worry about work performance; unsure whether concern is accurate.
Perception	Experience in the client's words, context, timing and impact.	No unusual experiences reported when asked.
Cognition	Relevant attention, memory, orientation or language, with task and limits.	Followed discussion; no formal testing undertaken.
Insight and understanding	Person's own understanding of experience and options, with example.	Linked worry with recent workload.
Judgment and decisions	Specific decision-making example; do not equate disagreement with impairment.	Considered options and chose to request a work review.

Do not infer: A single behaviour does not establish a disorder. Avoid unsupported labels such as "strange", "manipulative", "poor insight" or "poor judgment". If thoughts of harm emerge, make a separate, fuller assessment and follow local pathways.

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MSE write-up and practice

Describe what was assessed and what was not. State relevant limits such as language access, fatigue, distress, remote connection or time.

Worked example

Presentation: Clean work clothes; appeared tired. Behaviour: Engaged, with brief pauses before answering questions about sleep. Speech: Clear, normal volume, slightly slowed. Mood: “Worn out”; affect subdued and reactive. Thought process: Relevant and generally linear. Thought content: Worry about work performance; no unusual belief elicited in the topics explored. Perception: No unusual perceptual experience reported when asked. Cognition: Followed the discussion and recalled the agreed next step; no formal cognitive testing undertaken. Insight: Linked increased worry with recent workload. Judgment: Considered options for requesting a work review. This reflects one meeting, not a diagnosis or complete risk assessment.

From vague to useful

Vague: “Client seemed strange and had poor judgment.”

More useful: “Client paused for several seconds before answering, then described feeling watched at work. They were unsure what might explain the experience. When discussing next steps, they considered contacting a trusted colleague and chose to wait until after the meeting.”

The revised note separates observation, client report and a specific decision. It avoids stigma and leaves interpretation open.

Practice case: Sam’s first appointment

Sam attends after a referral for reduced sleep and missed deadlines. Sam says, “My mind keeps going over everything I might have done wrong.” They speak clearly, pause before answering and remain engaged. They describe feeling “tense and tired”. They report no unusual sensory experiences when asked. They can outline a recent work meeting in sequence and identify a supportive sibling. Sam has not yet been asked about self-harm, medication, substance use or recent health changes.

Task: Write a short case-history summary and MSE. Mark what is known, what remains unasked and what needs follow-up. Do not infer a diagnosis or risk level.

Learner notes

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Practice review and safe documentation

Use this page for teaching discussion, supervision or observed practice feedback.

Suggested learning points

A careful response can describe Sam's report of reduced sleep, repetitive worry and missed deadlines, alongside the observable pauses and continued engagement. It can note the sequence of the meeting, stated tension and tiredness, the response about perception, and the identified sibling support. It should state that safety, medication, substance use and health changes remain unassessed, then identify appropriate follow-up questions and consultation needs. There is not enough information to diagnose or characterise risk.

What is reported, observed and inferred?

Label each statement by source. Which interpretations need checking?

What follow-up questions matter?

Identify questions relevant to goals, health, safety, functioning or referral.

What action is agreed?

Name the action, who is responsible and by when. Include supervision or referral if needed.

Documentation reminders

Record the purpose, source, date and relevant context. Use neutral, specific language. Note uncertainty and limits. Do not use global low, medium or high risk labels as predictions. Focus on needs, immediate and longer-term safety, support and action in line with local guidance. Maintain confidentiality and store information through approved systems.

Further reading

- Mental Status Examination, StatPearls, NCBI Bookshelf
- NICE guideline NG225: self-harm assessment and care (UK guidance)

Disclaimer: "This is an educational training resource. It does not confer a qualification, registration, licence or competence to practise, and does not replace supervision, local law, service policy or professional judgement. For immediate danger, follow the emergency pathway for the client's location."

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